

Testicle removal surgery (orchidectomy) for testicular cancer

Surgery to remove the testicle is needed to diagnose cancer. It is also the main treatment when the cancer has not spread.

On this page

- [Removing the testicle \(orchidectomy\)](#)
- [Before your operation](#)
- [The operation](#)
- [After the operation](#)
- [Sex](#)
- [Fertility](#)
- [About our information](#)
- [How we can help](#)

Removing the testicle (orchidectomy)

As well as confirming a diagnosis, removing the testicle also removes the cancer. It is and the main treatment for testicular cancer that has not spread.

This may be the only treatment you will need if:

- the cancer has not spread outside the testicle ([stage 1](#))
- there is low risk of the cancer coming back (recurrence).

Your specialist doctor and nurse will explain why you need it and can talk through any concerns you may have. It is natural to be worried about having this operation and how it may affect your [body image](#), [sex life](#) or [fertility](#).

If your other testicle is healthy, removing one testicle will not affect your fertility or ability to get an erection.

Testicular implant or prosthesis

During the operation, the surgeon can put an artificial testicle into your [scrotum](#). This is called a testicular implant or prosthesis. If you are unsure about whether you want this at the time of your operation, you can have a prosthesis put in later. Your doctor will give you more details about the benefits and disadvantages of having an artificial testicle. They can explain how it will look and feel.

Before your operation

If you smoke, try to [stop or reduce how much you smoke](#) before your operation. This will help:

- lower your risk of chest problems, such as a chest infection
- help your wound heal after the operation
- reduce your risk of other health conditions.

The [NHS](#) has a lot of information and support to help you give up smoking. Look on the NHS website for the country where you live. Your GP or specialist nurse can also give you advice and information about local stop-smoking services.

Before your operation, you will meet a member of the surgical team and a specialist nurse or clinical nurse specialist (CNS). They will discuss the operation and you can ask any questions or talk over any concerns you have.

You will go to a pre-assessment clinic for some general checks such as [blood tests](#) and an ECG (a recording of your heart). This is part of the standard preparation before a general anaesthetic.

Fertility

You may be worried that the cancer will affect your ability to make someone pregnant (your fertility). Removing one testicle will not usually affect your fertility, as long as the remaining testicle is healthy.

But you will usually be asked if you want to store (bank) sperm before your operation, as long as it does not delay treatment too much. This is because you may need to have further treatment after your surgery, such as [chemotherapy](#). This may affect your fertility.

[Storing or freezing some of your sperm](#) means you can use it in the future to help you start a pregnancy. You may want to store your sperm even if you are not in a relationship.

You may be asked to give a sample of sperm to check your sperm levels first. If there is no sperm in the sample you give, it may be possible to take a sample from your testicle during surgery. There are also [other sperm collection techniques](#).

If your sperm count is low, or there is no sperm in the sample, that may be because the other testicle is small or making less sperm.

If you are offered further treatment, and have not stored any sperm, you can store sperm after surgery.

We have more information about [sperm banking](#).

If you are trans or non-binary, this may feel even more overwhelming. But it is important for you to have the option of preserving your fertility so you can make an informed decision. Your cancer doctor or specialist nurse can talk to you about this before your treatment starts.

We have more information about [transgender and non-binary people and cancer](#).

The operation

You have the operation as a day patient procedure. Before your operation, you will meet members of the surgical and nursing teams and the anaesthetist. The anaesthetist is the person who gives you the anaesthetic.

We have more information about [preparing for surgery](#).

You usually have the operation done under a general anaesthetic. But you may have it done under a local anaesthetic. Your anaesthetist and surgeon will discuss which is the best option for you.

The surgeon will make a small cut (incision) into the groin on the affected side. They will then push the testicle up from the [scrotum](#) and remove it through the incision.

After the operation

When you have recovered from the effects of anaesthetic, you will be able to eat and drink. The hospital staff will encourage you to get up and start walking around as soon as possible.

Possible complications of surgery

All operations have possible complications, and your surgeon will discuss these with you before. Possible complications will depend on the type of operation you have.

The most common complications after surgery are:

- a wound infection
- bleeding in the operation area
- developing a chest infection
- developing a blood clot.

The nurses will monitor you for these complications. Let them know straight away if you have:

- any bleeding, or feel unwell
- symptoms of an infection, such as a cough or feeling short of breath
- discharge from your wound
- swelling and redness in a limb.

As soon as you feel well enough and your doctor has checked you over, you can go home. This is usually the same day that you have the surgery. You will need someone to take you home and stay with you for the first 24 hours.

Recovering from surgery

You may have some side effects after surgery. There is often some discomfort, bruising and slight swelling around the scar for a couple of weeks. Taking regular painkillers will help this. The hospital will give you a temporary scrotal support garment. Or you may be advised to wear supportive underwear for about 2 weeks after your surgery. Wearing loose trousers may also help with any discomfort.

You may have numbness around the area, but this usually gets better slowly. But, for some people, there may be areas of numbness that never completely go away.

You usually have dissolving stitches. They can take a few weeks to dissolve. If you have non-dissolving stitches, they are usually removed about 5 to 10 days after your operation.

Driving and returning to work

Your doctor will advise that you only start driving when you feel comfortable enough. You should also be confident that you can do an emergency stop. Your specialist can give you more information about this.

Check with your insurance company to make sure that you are covered. You can discuss any concerns you have about driving after your orchidectomy with the [Driver and Vehicle Licensing Agency \(DVLA\)](#) if you live in England, Scotland or Wales. If you live in Northern Ireland, contact the [Driver and Vehicle Agency \(DVA\)](#).

Your specialist will also advise you not to do any heavy lifting for the first few weeks after your operation. The amount of time you need to take off work will depend on the type of work you do. Your hospital doctor can advise you more on this.

Sex

It is usually okay to have sex once your wound has healed. But you may not feel like having sex for a while after your surgery. This may happen if you are in some discomfort and feel anxious. You may be concerned about your sex life or sex drive after the testicle has been removed. Any negative feelings will usually get better gradually.

If difficult feelings or problems with your sex life continue, talk to your doctor or nurse. There are also organisations that might be able to help, such as the [Sexual Advice Association](#) and the [College of Sexual and Relationship Therapists](#).

We also have more information on [cancer diagnosis and relationships](#).

Fertility

You may be worried that the cancer will affect your ability to make someone pregnant (your fertility). Removing one testicle will not usually affect your fertility, as long as the remaining testicle is healthy.

But you will usually be asked if you want to store (bank) sperm before your operation, as long as it does not delay treatment too much. This is because you may need to have further treatment after your surgery, such as [chemotherapy](#). This may affect your fertility.

[Storing or freezing some of your sperm](#) means you can use it in the future to help you start a pregnancy. You may want to store your sperm even if you are not in a relationship.

You may be asked to give a sample of sperm to check your sperm levels first. If there is no sperm in the sample you give, it may be possible to take a sample from your testicle during surgery. There are also [other sperm collection techniques](#).

If your sperm count is low, or there is no sperm in the sample, that may be because the other testicle is small or making less sperm.

If you are offered further treatment, and have not stored any sperm, you can store sperm after surgery.

We have more information about [sperm banking](#).

If you are trans or non-binary, this may feel even more overwhelming. But it is important for you to have the option of preserving your fertility so you can make an informed decision. Your cancer doctor or specialist nurse can talk to you about this before your treatment starts.

We have more information about [transgender and non-binary people and cancer](#).

About our information

References



Below is a sample of the sources used in our testicular cancer information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

The British Association of Urological Surgeons (BAUS) Guidelines on Testicular Cancer. March 2015. Available from: https://www.baus.org.uk/professionals/sections/testicular_cancer.aspx (accessed April 2022).

European Association of Urology (EAU) Guidelines on Testicular Cancer 2022. Available from: <https://d56bochluxqnz.cloudfront.net/documents/full-guideline/EAU-Guidelines-on-Testicular-Cancer-2022.pdf> (accessed April 2022).

European Society for Medical Oncology (ESMO). Testicular seminoma and non-seminoma: ESMO Clinical Practice Guidelines for diagnosis, treatment and follow-up. Last updated 2022.

European Society for Medical Oncology (ESMO). Testicular seminoma and non-seminoma:

ESMO Clinical Practice Guidelines for diagnosis, treatment and follow-up. Last updated 2022. Available from: [https://www.annalsofoncology.org/article/S0923-7534\(22\)00007-2/fulltext](https://www.annalsofoncology.org/article/S0923-7534(22)00007-2/fulltext) (accessed April 2022).

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, [Dr Ursula McGovern](#), Consultant Medical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

Reviewed: 01 May 2022 | Next review: 01 May 2025

ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information.](#)

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help